What Long-Term Care Medicaid Looks At Besides the Bank Balance

A plain-English guide for families paying for care now.

Someone told you Mom has too much money for Medicaid. Maybe it was said kindly, in a hallway, by a person who had never seen one of her statements. They may be right or they may not. Either way, it's an answer to a question nobody has properly asked yet.

This guide explains, in general terms, the parts of a person's situation that long-term care Medicaid looks at, and what your family can gather and ask about each. It explains. It doesn't say where Mom stands.

How long-term care Medicaid works

Medicare doesn't pay for long-term care: the ongoing help with dressing, bathing and the other basic tasks of everyday life that Medicare.gov also calls custodial care. [1]

Medicaid is a different program. It is run jointly by the federal government and the states, and unlike Medicare it does pay for custodial care, in nursing homes and at home. What it covers differs from place to place: a service available in one state may not be available in another. [2] [3]

It also doesn't work like ordinary health insurance. To be eligible, a person has to meet requirements that include income and assets within the levels her state uses, and the state then decides separately whether she qualifies for long-term care services. The states run the program day to day, take the applications and make the decisions. [2] [4]

- Write down the state Mom lives in and where she is being cared for. Both come into any conversation about Medicaid.
- When someone says “Medicaid pays for that” or “Medicaid won't pay for that,” note who said it, and ask which state's program they mean.

Money isn't the whole test

Some requirements have nothing to do with finances. Medicaid.gov says people generally must be residents of the state where they receive Medicaid, and either citizens of the United States or certain qualified non-citizens. [5]

Then there is the care itself. For long-term care, a nurse or social worker in the state evaluates the person's needs and decides whether she needs long-term care services. The federal long-term care site is plain that this stands apart from money: a person who doesn't meet the care criteria isn't covered for long-term care services, whatever her finances. [6]

How much need is enough is not the same everywhere. The threshold for that level of care varies by state. [7]

- Write down the help Mom gets in a day, who gives it, and since when. If the facility or her doctor has assessed her care needs, note who did it and when.
- A question to bring: who assesses the level of care in Mom's state, and at what point?

Income and assets are lists, not one number

When a state looks at income, it counts some of it but not all of it, and the amount a person can have varies by state. The kinds of income it looks at include Social Security, veterans benefits, pensions, and interest and dividends. [6]

Assets work the same way. Some are counted and others are excluded, and the federal site says the assessment can be fairly complex, depending on how much and what kind of assets a person has. It also reminds readers that many assets are not counted at all. [6]

The words you'll hear for this are countable, meaning assets whose value is counted in determining financial eligibility for Medicaid, and exempt or non-countable, meaning assets whose value is not. [8]

The home has questions of its own. A state may ask a homeowner to document the home's current fair market value and any loans against it, and states have varying rules about home equity. [4] [7]

That is why “she has too much money” can't be settled from a bank balance. Which list each thing belongs on, under the rules in Mom's state, is the part a professional does.

- List what Mom receives each month and from whom, and what she owns, with whose name is on each. Mark every line “seen on paper” or “from memory.”
- Put retirement accounts, life insurance and annuities on their own lines, with the company and the owner. How each is treated is a question for a professional who knows her state.
- Don't sort anything into countable or exempt yourself, and don't add up a total to hold against a number you've heard.

A spouse, and what has changed hands

If Mom is married, more rules come into it. Medicaid.gov describes provisions meant to keep the spouse who is still living at home from being left with little or no income or resources. Under them, it says, a certain amount of the couple's combined resources is protected for that spouse. The amounts are updated from year to year, and how any of this may apply to one couple is for a professional who knows their state to say. [9]

Then there is history. A state may ask an applicant to explain what happened to assets whose value went down a lot, and it will want to know whether any were given away. The federal glossary describes a transfer of assets as giving away property for less than it is worth, and says transfers made during what is called the look-back period can mean a stretch of time when Medicaid doesn't pay for long-term care services. [4] [8]

How long that period is, what counts as a transfer and what follows from one all depend on the state and the details. So this guide says nothing about moving, giving or retitling anything. Those are decisions to make with an elder law attorney or your Medicaid planner, once they've seen the whole picture.

- Note whether Mom has a spouse, where the spouse lives, and the date she moved into care.
- Write down any gift, transfer or name added to an account or a deed that anyone remembers, with rough dates and who was involved.
- If something has already changed hands, write it down plainly. It's a line on the list, not a verdict.

What this means for your family

- “Too much money” is one person's summary of one part of the picture. The sources above describe Medicaid looking at a person's care needs, her income, her assets, her home, whether she is married, what has changed hands, and the rules in her state. Each of those is a part of Mom's picture to get onto paper.
- Your part is to get each of those onto paper, with what you've seen kept apart from what you've only been told.
- Your Medicaid planner reviews what you've gathered against the rules that apply in Mom's state and care setting, and separates what can be handled administratively from what needs an elder law attorney or a licensed financial professional.
- Your state's Medicaid office is, in the federal site's words, the best source of information about how to qualify in your state (source 2 below).

What this guide doesn't do

It doesn't say what the rule is in Mom's state, whether she qualifies, or what your family should do, and it isn't legal, financial or tax advice. The general statements come from the sources below. The suggestions about what to write down are ours.

Ready for the next step?

You know the parts of Mom's situation that Medicaid looks at, and what to gather for each. The planning interview asks about those parts in turn, so nothing important is left out, and gives you a free planning report you can bring to any conversation about paying for care.

Get Your Free Planning Report ›

WHERE THIS COMES FROM

1. Medicare.gov (Centers for Medicare & Medicaid Services), Long Term Care Coverage
2. Administration for Community Living (LongTermCare information), Medicaid
3. Administration for Community Living (LongTermCare information), Medicaid Long-term Care Services
4. Administration for Community Living (LongTermCare information), Applying for Medicaid
5. Medicaid.gov (Centers for Medicare & Medicaid Services), Eligibility Policy
6. Administration for Community Living (LongTermCare information), Medicaid Eligibility
7. Justice in Aging, Medicaid Home and Community-Based Services for Older Adults: A Primer
8. Administration for Community Living (LongTermCare information), Glossary
9. Medicaid.gov (Centers for Medicare & Medicaid Services), Spousal Impoverishment